

REVIEW

Developing a Counselling and Psychotherapy Competency Framework for Addictions: A Systematic Review

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ABSTRACT

Background: Adults with addictions may turn to professional counsellors and psychotherapists for help. Addictions work comprises a specialist counselling skillset. This article summarises the literature review and analysis that underpins the British Association for Counselling and Psychotherapy's (BACP) Addictions Competence Framework.

Aims: This study aimed to identify the specialist knowledge, skills and experience associated with effective counselling for adults seeking help with addictions, whether active or in recovery.

Materials and Methods: We systematically reviewed empirical research and wider literature. The EBSCO database was searched following the Preferred Reporting Items for Systematic Reviews (PRISMA) guidelines, using a range of search terms and appropriate inclusion and exclusion criteria. Relevant data were extracted, reviewed and thematically synthesised.

Results: Forty-seven relevant articles were identified. Five superordinate themes were developed: (1) specialist addiction knowledge; (2) assessment and treatment planning; (3) training, supervision and professional practice; (4) client-focused approach; (5) specific therapeutic techniques. Findings from this study indicate that addictions counselling work is enhanced when therapists have knowledge of different addiction models and are able to offer suitable interventions for the pre-treatment phase, relapse prevention and for supporting the recovery process.

Discussion: Effective therapy adopts an integrative, client-focused approach that fosters safety and collaboration. It focuses on managing cravings/urges, addressing emotional concerns and helping clients challenge self-defeating thoughts and beliefs. Techniques associated with effective addictions counselling include psychoeducation, motivational interviewing and mindfulness-based strategies. Delivery style is characterised by empathy and a non-confrontational stance.

Conclusion: This review identifies specialist knowledge, skills and experience associated with effective counselling for adults seeking help with addictions and provides evidence to underpin the BACP's Addictions Competence Framework.

1 | Introduction

Many adults experiencing addiction may seek help from professional counsellors and psychotherapists (Capuzzi and Stauffer 2024; Cavaola and Smith 2020; Lassiter and Culbreth 2017). Addiction competence frameworks for

therapists¹ exist in the USA and internationally, for example from the Substance Abuse and Mental Health Services Administration (2015) and from the United Nations Office on Drugs and Crime (2020). The UK has some general workforce standards, such as from Drug and Alcohol National Occupation Standards (2008) and from the Department of Health and

Implications for Practice and Policy

- This paper summarises the systematic review that underpins BACP's Addictions Competence Framework (2024), identifying the key knowledge and skills associated with effective addictions counselling for adults.
- Five superordinate themes were identified: specialist addiction knowledge, assessment and treatment planning, training/supervision and professional practice, a client-focused approach and specific therapeutic techniques. These themes highlight addictions work as a specialist area requiring specialist knowledge, skilled practice and ongoing professional development.
- Findings show that addictions counselling is strengthened when therapists understand different addiction models and can support clients at different stages (pre-treatment, relapse prevention and recovery). Effective therapy is integrative, empathic and collaborative, with a clear focus on building trust and therapeutic relationship.
- The review highlights the importance of trauma-informed and culturally responsive practice, recognising the impact of diversity, co-occurring difficulties and wider systemic factors. The framework outlines key useful interventions (e.g., psychoeducation, motivational interviewing and mindfulness) to support clients with cravings/urges, emotional distress and longer-term recovery.

Social Care/NHS England (2024). However, at the time the current study commenced, there were no up-to-date competence frameworks to guide UK therapists working with adults with addictions.

In 2018, in response to a need identified among its members, the British Association for Counselling and Psychotherapy (BACP) met with several specialist organisations within the addictions field to explore the feasibility of developing a competence framework for working with addiction in counselling/psychotherapy that would define the core knowledge, skills and abilities required by therapists to work safely and effectively with adults with addictions. The stakeholders agreed the framework should also enable appropriate training and qualifications to be developed and promoted to a national standard. BACP led the development of this framework.

Several working groups were involved in initial framework development: (1) a Core Group (CG) which managed the process and consisted of BACP staff, including members from the professional standards and research departments, and external expert academics and clinical practitioners; and (2) an Expert Reference Group (ERG) of stakeholders and addiction specialists who provided external, independent advice and support, and who helped guide the work of the CG in the development and implementation of the competence framework.

This article presents the method, findings and analysis of the underpinning literature review conducted by the CG to distil and integrate common approaches that promote helpful change and recovery within addictions treatment, plus

therapists' education, training and professional development requirements. The analysis informed further interpretive processes undertaken by the CG and ERG to produce the final framework (British Association of Counselling and Psychotherapy 2024). Recovery is 'a dynamic process characterized by increasingly stable remission resulting in and supported by increased recovery capital and enhanced quality of life' (Kelly and Hoepfner 2014, p. 9).

2 | Method

2.1 | Search Strategy and Eligibility Criteria

Preferred Reporting Items for Systematic Reviews (PRISMA) guidelines (Page et al. 2020) were followed. Searches and analysis were conducted between February 2021 and April 2022 using EBSCO's Psychology and Behavioural Sciences Collection. To identify as much relevant research as possible, 21 primary terms² and 31 secondary terms³ were searched. A third set of search terms was used to refine the scope of the literature search and included Substance, Alcohol and Drug*. English language articles published between 1996 and 2021 were included if they described knowledge, skills and training related to therapeutic practice with individual adult clients/participants with substance, alcohol, sex or gambling addictions. Also included were relevant discussion articles, clinical reports, systematic or literature reviews (including meta-analyses) relating to addiction. Excluded were articles relating to: (1) addiction to video games or food; (2) pharmacological interventions; (3) child, young people, families, couples and group interventions; and (4) online and/or digitally mediated interventions.

2.2 | Screening Procedures

The search terms initially returned a total of 11,128 articles. After duplicates were removed ($n = 2033$), 9095 possible articles remained and were shared out equally between four researchers (FM, WF, JP and TP). Titles and abstracts were screened for inclusion/exclusion criteria, and 8059 articles were removed after this screening procedure. Full texts of the remaining 1036 articles were retrieved and considered for inclusion. A further 838 articles were excluded. The remaining 198 articles were assessed rigorously against eligibility criteria. Another 151 articles were excluded at this stage. Forty-seven articles remained that met inclusion criteria. The full screening process is presented in Figure 1 below.

3 | Systematic Review of the Included Articles

Three researchers (FM, WF and GS) extracted relevant data from the remaining 47 articles. The research team used Microsoft Excel to organise and collate data as part of the extraction process. Data were collected using predefined criteria: Basic article information (author(s), year of publication, title, DOI); Study design and methodology; Participant demographics (e.g., age, gender, addiction type, relevant health conditions); Intervention details, such as type, format, duration, and setting; and Reported outcomes, both quantitative and qualitative. Each

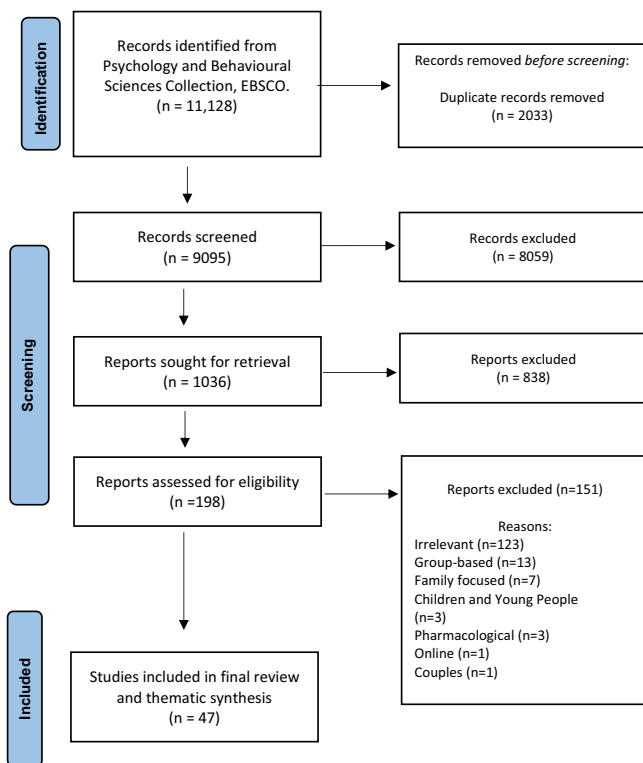


FIGURE 1 | PRISMA flow chart.

paper was independently reviewed by three members of the research team, who applied descriptive coding to extract key information. Following this, the reviewers met to compare interpretations and addressed any discrepancies through collaborative discussion. The overall review approach was informed by the principles of systematic reviews (Higgins et al. 2019; Moher et al. 2009), and for the qualitative studies specifically, the thematic extraction process followed the approach outlined by Thomas and Harden (2008). The overall threshold for relevance was based on addiction treatment-related competencies. Thematic synthesis generally involves three overall stages: first, coding the text line by line; second, developing descriptive themes; and finally, generating analytical themes that extend the interpretation (Thomas and Harden 2008). Table 1 provides an overview of key findings.

3.1 | Characteristics of Included Articles Are Summarised Below

3.1.1 | Study Locations

Thirty-four (73%) studies were undertaken in the USA; 4 (8.5%) in the UK; 4 (8.5%) in Australia; and 1 (2%) from each of the following countries: Canada, Israel, Norway, Switzerland, and Spain.

3.1.2 | Study Type

Twenty-four (51%) articles included data from quantitative studies, including treatment trials, single-arm outcome studies

and surveys. Of these, five were randomised controlled trials, two were non-controlled randomised trials, and the remaining seven were outcome and survey studies. Ten (21%) were qualitative explorations. Of these, three were case studies. Of the seven participant interview studies, three were analysed with thematic analysis, two with grounded theory, one with content analysis and one with phenomenological hermeneutic enquiry. Twelve (25.5%) articles were literature reviews that included discussion of competency-relevant information. One (2.5%) article was an editor's brief introduction to a special issue that included competency-relevant information.

3.1.3 | Study Focus

Eleven (52%) articles focused specifically on counsellor competences. Of these, five focused on generic addiction competences; the remaining six focused on chemical addiction/substance misuse/alcohol and drug misuse competences. Eight (17%) focused on dual diagnosis or co-morbidity treatments. Of these, one focused on psychosis and drug use; two on personality disorders and substance misuse; two on PTSD and drug/alcohol use; one on depression and substance misuse; one on serious mental health difficulties and substance misuse; and one on substance misuse and gambling. Sixteen (34%) focused on substance misuse/drug and alcohol treatment. Six (13%) articles focused on alcohol misuse treatment. Four (8.5%) focused on sexual addictions/compulsivity; and two (4%) focused on gambling.

An inductive approach consistent with systematic review methods, incorporating thematic synthesis, was used to identify practice-relevant competences from heterogeneous data sources.

- The 47 articles were distributed among three researchers (FM, WF and GS). Each researcher independently reviewed articles, actively identifying and reflecting on practice implications relevant to addiction counselling competencies and training. The researchers then convened to compare notes, share impressions, and collectively examine patterns in their findings.
- Each researcher thoroughly appraised their assigned articles and manually coded significant components or factors that could inform competencies or effective practices in addiction work. Through discussion, the researchers collaboratively generated and agreed on 59 overall codes. These were:
 1. acceptance and/or compassion
 2. aetiology and theories of addiction
 3. attending skills
 4. case management
 5. challenging maladaptive beliefs/values/behaviours
 6. client attachment styles
 7. client characteristics
 8. client motivation
 9. client perceptions
 10. client resistance
 11. client safety
 12. client-led approach
 13. common factors

TABLE 1 | Characteristics and results/implications from studies included in review.

Article No.	Authors and year of study	Study type	Key conclusions or practice implications relevant to competencies and/or training
1	Thombs and Osborne (2001)	Quantitative, survey with cluster analysis	Practitioners have divergent perspectives on addiction treatment. Some favour a moral-disease model and others, a pluralistic, psychosocial approach. Training(s) may wish to adopt non-traditional perspectives and explore multiple treatment options, including motivational interventions and practical/client-focused solutions
2	Pavlick et al. (2009)	Quantitative, survey with descriptive stats	Focus on managing cravings. Training(s) would benefit from including craving instruments, exposure interventions, and coping skills, encouraging clients to avoid craving situations and educating them about cravings
3	Martino et al. (2006)	Quantitative, Randomised Control Trial (RCT)	Those with psychotic diagnoses may need more extensive, combined interventions for lasting outcomes. Motivational interviewing may not be as effective for dually diagnosed clients, though motivation plays a key role in recovery
4	McGovern et al. (2015)	Quantitative, Randomised Control Trial (RCT)	Integrated Cognitive Behaviour Therapy (ICBT) may offer better drug-related outcomes for patients with post-traumatic stress disorder and substance use disorder patients compared to drug-focused counselling. ICBT is suitable for community practitioners and routine practice settings
5	Kasarabada et al. (2001)	Quantitative, instrument validity testing with factor analysis	48-item measure encompasses major techniques used in drug-user treatment and for assessing therapeutic approaches
6	Lanza et al. (2014)	Quantitative, Randomised Control Trial (RCT)	ACT may be a suitable alternative to CBT for treating drug abuse and related mental disorders for incarcerated women
7	Fraumeni-McBride (2019)	Literature review	Pornography addiction is linked to negative emotional, psychological, and social outcomes. Practitioners are encouraged to reduce these negative outcomes and manage compulsions. Mindfulness and ACT, which raise awareness of thoughts, emotions, and behaviours and reconstruct beliefs, may be beneficial
8	Lee et al. (2013)	Literature review	Training programmes benefit from focusing on assessing, diagnosing, case managing, treatment, relapse prevention, and recovery tools. Reflective and experiential learning are beneficial instructional strategies
9	Lee (2014)	Qualitative, interviews with thematic analysis.	Addiction courses requiring practice hours should cover addiction types, substances, administration routes, assessment, diagnosis, treatment approaches, co-occurring diagnoses, specific populations, and family issues. Experiential activities are encouraged to address addiction stigma

(Continues)

TABLE 1 | (Continued)

Article No.	Authors and year of study	Study type	Key conclusions or practice implications relevant to competencies and/or training
10	Moro et al. (2016)	Qualitative, content analysis	Counselling educators working with addictions might consider alternative professional development routes like webinars, newsletters, online and in-person training, and other resources
11	Toriello and Strohmer (2004)	Quantitative, experimental with crossed factorial analysis	Counsellors are encouraged to work closely with non-verbal behaviours
12	Giordano et al. (2016)	Quantitative, survey with descriptive analysis	Practitioners should aim to integrate trauma recovery and empowerment models into their work. Conceptualisation can help clients manage emotions, identify triggers, and process trauma. Practitioners are advised to employ self-care strategies to prevent vicarious trauma and burnout
13	Wheeler et al. (2014)	Quantitative, survey with descriptive analysis and ANOVA	Practitioners are recommended to receive substance abuse training to enhance knowledge and attitudes towards affected populations. Services should ensure practitioners have access to expertise, service links, and research opportunities. Support and mentoring can ensure sustainable skills to meet client needs
14	Jones et al. (2016)	Qualitative, interviews and grounded theory analysis	A non-confrontational approach, affirmation, and highlighting discrepancies between beliefs and behaviour appear to contribute to the efficacy of motivational interventions
15	Vujanovic et al. (2017)	Literature review and theory	Practitioners may implement a transdiagnostic model for treating substance abuse disorder with depression, utilising cognitive-affective processes like rumination, emotion regulation, and distress tolerance. Sessions focus on substance use reduction, improving negative affect through cognitive skills training, increasing meta-cognitive awareness and engaging in value-driven behaviours. Emotion regulation strategies and distress tolerance skills including mindfulness and physical exercise can be helpful
16	Shakeshaft et al. (2002)	Quantitative, Randomised Control Trial (RCT)	Clients are encouraged to take responsibility for their behaviour change, with outcomes measured to track progress. Attendance at counselling sessions, rather than intervention type, is key for reducing alcohol misuse
17	Brookes and Penn (2003)	Quantitative, comparative study using hierarchical linear modelling	Practitioners are advised to measure substance use frequency, quantity, patterns, and impact on quality of life
18	Clark (2009)	Literature review	Positive treatment outcomes are linked to quality of client-practitioner relationship, facilitated by the therapeutic alliance. Motivational interviewing is effective for managing resistance and raising client awareness. Practitioners are urged to use warmth, empathy, and congruence, and be mindful of how substance use may hinder engagement

(Continues)

TABLE 1 | (Continued)

Article No.	Authors and year of study	Study type	Key conclusions or practice implications relevant to competencies and/or training
19	Thornton et al. (2003)	Quantitative, Randomised Control Trial (RCT)	Practitioners may wish to tailor strategies to client characteristics as certain traits may favour high or low structured interventions
20	Gainsbury (2017)	Literature review	Incorporating cultural competence into addiction treatment programmes improves outcomes. Key areas to consider include: • Community: education, social support • Clinician attitudes, language • Treatment location, cultural adaptations • Client attitudes towards treatment, language
21	Adrian (2001)	Literature review and theory	Outcomes in addiction treatments vary depending on client concerns, intervention type, and treatment timeframes
22	Osborn (1997)	Quantitative, survey with multiple regression analysis	Practitioners need to be aware that the disease concept of alcoholism may conflict with Solution-Focused Brief Therapy (SFBT)
23	Kvamme et al. (2015)	Qualitative, interviews and hermeneutical phenomenological analysis	Unpredictable cravings are a risk factor for relapse. Practitioners should recognise client narratives and experiences to help prevent relapses. Practitioners should reflect on their experiences and subjectivity, considering how these may influence their work with clients
24	Rothrauff-Laschober et al. (2013)	Quantitative, survey with mixed-effects models	Practitioners should engage in ongoing professional development for understanding and treating complexities surrounding substance abuse disorders. Clinical supervision helps ensure positive client outcomes
25	Gottheil et al. (2002)	Quantitative, non-RCT, treatment comparisons and <i>t</i> -tests	Both high- and low-structure interventions have proven effective. Practitioners can improve outcomes by matching clients to interventions based on pre-treatment addiction severity and psychological concerns of clients
26	Weegmann and Khantzian (2011)	Qualitative, case study method	Practitioners may view addiction as a condition characterised by disordered emotions, relationships, and behaviours, often linked to trauma, grief, and loss. Recovery requires comfort and safety for the client, especially in early treatment. Counselling helps restore or establish a lost or missing narrative about the self and others
27	Aiach Dominitz (2017)	Qualitative, case study method	Gestalt Therapy (GT) encourages awareness and contact. It should be used as a therapeutic process rather than a toolbox, helping clients develop authentic relationships and mindful, adaptive self-awareness

(Continues)

TABLE 1 | (Continued)

Article No.	Authors and year of study	Study type	Key conclusions or practice implications relevant to competencies and/or training
28	Dakwar and Levin (2013)	Quantitative, descriptive analysis and <i>t</i> -tests	Mindfulness-based psychotherapy shows promise for substance abuse disorders. Practitioners can incorporate meditation into treatment plans and/or use a patient-directed approach to cultivate mindfulness. Individual psychotherapy is more responsive and adaptable to patient needs compared to group-based therapy
29	Joe et al. (2009)	Quantitative, treatment trial with multivariate analysis of covariance	Strong client-therapist rapport can reduce relapse risk and better address emotional needs. Awareness of cocaine use is crucial, especially during methadone treatment, as it affects behavioural outcomes like readiness to change and motivation
30	Dimaggio et al. (2015)	Qualitative, case study method	Metacognitive Interpersonal Therapy addresses personality disorders and substance abuse by focusing on maladaptive interpersonal schemas, metacognitive dysfunctions, and dysfunctional regulatory strategies. This approach enhances social relationships, emotional awareness, and regulatory skills
31	Graham et al. (2016)	Quantitative, Randomised Control Trial (RCT)	Clients found Brief Integrated Motivational Intervention (BIMI) helpful for discussing experiences with substance use and mental health impacts. Practitioners can use BIMI to develop coping strategies and improve client engagement
32	Sannibale et al. (2013)	Quantitative, Randomised Control Trial (RCT)	Cognitive behaviour therapy for alcohol use disorders benefits individuals with co-existing post-traumatic stress disorder (PTSD). Exposure therapy is particularly effective for treating PTSD. These therapies produce significant, lasting improvements for those with dual disorders
33	Lawson et al. (2011)	Literature review and theory	Practitioners may combine motivational interviewing (MI) and developmental counselling and therapy (DCT) for a holistic approach to addiction treatment to help clients explore their recovery identity. MI strategies can focus first on maintenance, then DCT can address emotions and behaviours for enduring recovery
34	Spooner and Lyddon (2007)	Literature review and application to practice	Group social interaction, acceptance and support can help people recovering from sexual addiction. Group work encourages patience, tolerance, listening, and being present. Adequate training and supervision are necessary, especially for people with substance abuse and trauma

(Continues)

TABLE 1 | (Continued)

Article No.	Authors and year of study	Study type	Key conclusions or practice implications relevant to competencies and/or training
35	Hagedorn (2009)	Quantitative, survey with descriptive statistics and factor analysis	Practitioners should understand sexual addiction and gain relevant experience at graduate level. Working with clients with sexual additions requires establishing rapport with family and/or significant others, obtaining medical history, assessing for self-harm or harm to others, clarifying goals, conducting diagnostic interviews, administering assessments, determining addiction severity, educating on addiction types, promoting self-help groups, implementing cognitive counselling techniques, and preparing clients for termination of counselling
36	Kasl (2002)	Literature review and application to practice	To be trusted, professionals working with lesbian women need to demonstrate their understanding of homophobia. Training should cover issues faced by lesbian women and why sexual addiction is overlooked. Practitioners should demonstrate acceptance, engagement, and awareness of cultural discrimination
37	Krebs et al. (2018)	Quantitative and meta-analysis	Practitioners are advised to set realistic goals and integrate readiness to change into intervention resources, such as self-help materials, health apps, and online treatments. Practitioners should be experienced in supporting underserved populations
38	Southern (2013)	Editorial commentary	This paper mainly highlights the importance of 'competency-based' training. The editor presents several articles in this specific issue of the journal, covering topics such as counsellor training in addictions counselling, offender counselling, and competency-based education
39	Watts et al. (2018)	Quantitative, self-report measures, 2-tailed Pearson correlation and multiple regression analysis	Practitioners should recognise childhood maltreatment commonly experienced by clients with substance abuse issues. Establishing ongoing dialogue about therapeutic goals ensures mutual agreement on behavioural changes, supporting trust and working alliance
40	Steinglass (2009)	Literature review and application to practice	Providing intervention for substance misuse can involve the family around the client. Practitioners should remain neutral, use non-judgmental language, understand individual and family beliefs about substance use, and create a collective action plan for addressing addictive behaviour
41	Mooney et al. (2019)	Quantitative, self-report measures, Wilcoxon signed rank tests	A brief relational psychodynamic approach may benefit difficult-to-treat gamblers, especially when CBT has failed, and especially for female clients
42	Jones and Branco (2020)	Literature review and application to practice	Substance use disorder counsellors may face burnout, vicarious trauma, and secondary traumatic stress, leading to impairment. Trauma-informed supervision can help reduce these risks

(Continues)

TABLE 1 | (Continued)

Article No.	Authors and year of study	Study type	Key conclusions or practice implications relevant to competencies and/or training
43	Edwards and Loeb (2011)	Qualitative, interviews and grounded theory analysis	Helpful factors in counselling for people who are drug users on low income include experiencing togetherness in the therapeutic relationship, the practitioner responding attentively and professionally, the client valuing the practitioner, and client active engagement
44	Daepfen et al. (2010)	Quantitative, speech analysis and bivariate analysis	Client change talk in MI is associated with client-practitioner relationship. Counsellors with superior MI skills achieved better outcomes with patients regardless of stage of readiness to change. Those with inferior skills were mostly effective with patients ready to change
45	Oakes et al. (2020)	Qualitative and literature review, interview and unspecified thematic extraction	Practitioners should help clients with addiction to reduce distress, increase motivation, enhance readiness for change, and explore options. Moments when clients seek support are opportunities for reflection without delving into triggering storytelling. Practitioners should acknowledge client problems and use motivational strategies
46	Dwyer-Clonts et al. (2017)	Literature review and application to practice	Transtheoretical brief interventions include FRAMES (feedback, responsibility, advice, menu of options, empathy, self-efficacy)
47	Allen and Olson (2016)	Qualitative, interviews and thematic process and contextual examination	Therapeutic alliance is associated with treatment efficacy for substance use disorders. Practitioners should use exploration, reflection, and attentiveness, and facilitating emotional expression. Flexibility, negotiation, and motivation positively impact alliance. Incorporating motivational interviewing can help clients feel understood and work towards a common goal

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|---|---|
| 14. comorbidity | 34. pharmacology |
| 15. coping skills | 35. pre-treatment and assessment |
| 16. counsellor characteristics | 36. prevention |
| 17. cravings | 37. process |
| 18. creative methods | 38. professional practice |
| 19. crisis management | 39. psychoeducation |
| 20. culturally aware practice | 40. reality testing |
| 21. depression and/or anxiety | 41. recovery |
| 22. developing client self-awareness | 42. relapse |
| 23. diagnosis | 43. self-esteem, shame, and guilt |
| 24. empathy | 44. settings and contexts (e.g., outpatients, prison, community services) |
| 25. goal setting | 45. solution-focused approach |
| 26. high structure behavioural approaches (directive) | 46. specialist knowledge |
| 27. integrated treatment models | 47. specific populations |
| 28. low structure facilitative approaches (non-directive) | 48. standards and competencies |
| 29. mindfulness | 49. strengths-based approach |
| 30. motivational interventions | 50. supervision |
| 31. non-confrontational stance | 51. systemic factors (e.g., family, society, relationships) |
| 32. outcomes | 52. therapeutic relationship (e.g., transference, alliance) |
| 33. peer support | |

TABLE 2 | Superordinate themes and sub-themes.

Superordinate themes	Subthemes
1. Specialist addiction knowledge	1. Knowledge of addictions and models of addiction [Key articles: 4, 8, 9,10,12,13, 17,18, 21, 22, 26, 32, 33, 45] 2. Knowledge of treatment process [Key articles: 5, 8, 9, 10, 13, 15, 16, 17, 18, 21, 22, 26, 32, 33, 45] 3. Knowledge of treatment options [Key articles: 6, 9, 10, 13, 15,16, 21, 23, 26, 32, 33, 45]
2. Assessment and Treatment Planning	1. Pre-treatment and assessment [Key articles: 2, 8, 9, 10, 12, 13, 15, 19, 35] 2. Treatment planning [Key articles: 6, 8, 9,10, 13, 15, 19, 21, 35, 45] 3. Relapse prevention [Key articles: 9, 13, 15, 16, 18, 19, 21, 23, 28, 30, 35, 45] 4. Understanding and managing recovery [Key articles: 6, 13, 15, 16, 18, 26, 28, 30, 35, 45]
3. Training, development and professional practice	1. Appropriate training and CPD [Key articles: 1, 5, 8, 9, 10, 12, 13, 24, 38] 2. Supervision [Key articles 1,5, 8,9, 10, 12, 13, 20, 24,26, 37, 38, 42] 3. Working with diverse clients [Key articles: 9, 10, 12, 13, 20, 36, 37]
4. Client-focused approach	1. Trauma-informed approach [Key articles: 4, 6,12, 23, 26] 2. Focus on cravings and urges [Key articles: 2, 6, 7, 13, 14, 15, 16, 17, 23, 28, 44]
5. Specific therapeutic techniques	1. Therapist stance and characteristics [Key articles: 1, 5, 6, 10, 11, 14, 15, 18, 19, 27, 29, 39, 40, 41, 43, 46, 47] 2. Practical therapeutic skills and techniques [Key articles 3, 6,11, 14, 15, 16, 17, 18, 19, 23, 27, 28, 31, 34, 36, 47] 3. Structured behavioural approach [Key articles: 11, 13, 14, 15, 16, 17, 18, 19, 25, 28] 4. Reflective approaches [Key articles: 6, 11, 13, 14, 15, 18, 19, 25, 28, 29]

- 53. therapist flexibility
- 54. treatment planning/options (e.g., 12 step, psychodynamic, residential)
- 55. training and continuous professional development (CPD)
- 56. use of immediacy
- 57. working with ambivalence
- 58. working with trauma
- 59. working with withdrawal

- Generating, reviewing and defining themes involved the researchers collaborating to discuss and examine the codes in greater detail and generating, developing and defining potential descriptive and analytical themes. After in-depth discussion, consensus was reached on five superordinate themes with subthemes, as presented in Table 2 and Figure 2. Key articles from Table 1 that substantially address the subthemes are included in brackets.

4 | Findings and Discussion

This systematic review and thematic synthesis identified key themes in addiction counselling, highlighting the need for specialist knowledge, comprehensive assessment and treatment planning, continuous professional growth, client-centred approaches, and effective therapeutic techniques. These elements are vital for evidence-based, client-focused addiction treatment that supports long-term recovery. We will now review the key themes in more detail.

4.1 | Theme 1: Specialist Addiction Knowledge

4.1.1 | Subtheme 1: Knowledge of Addiction and Models of Addictions

This review indicated that effective addictions work requires specialist knowledge encompassing a comprehensive understanding of various addiction models, including medical, attachment, cognitive-behavioural, humanistic, trauma-informed, and social learning perspectives (Thombs and Osborne 2001). A robust grasp of aetiological factors underlying addiction is helpful, including neurobiological mechanisms, environmental influences, and the role of adverse childhood experiences (Weegmann and Khantzian 2011). Physiological and psychological effects of various substances and addictive behaviours should be understood. Therapists need to be appropriately trained and experienced to support adults with complexities arising from comorbid addiction and PTSD, depression, or anxiety (McGovern et al. 2015; Vujanovic et al. 2017). Teaching around addiction at individual, familial, and societal levels should include detrimental psychological impacts of poverty and housing instability. Therapists should understand the consequences of other systemic inequalities, such as access to mental health services, social care, and distribution of criminal justice proceedings. Finally, practitioners are encouraged to seek out current knowledge and evolving therapeutic approaches in substance use and addictive behaviours (Lee 2014). A commitment to lifelong learning, evidence-based practice, and interdisciplinary collaboration enables therapists to effectively support clients in navigating addiction and recovery.

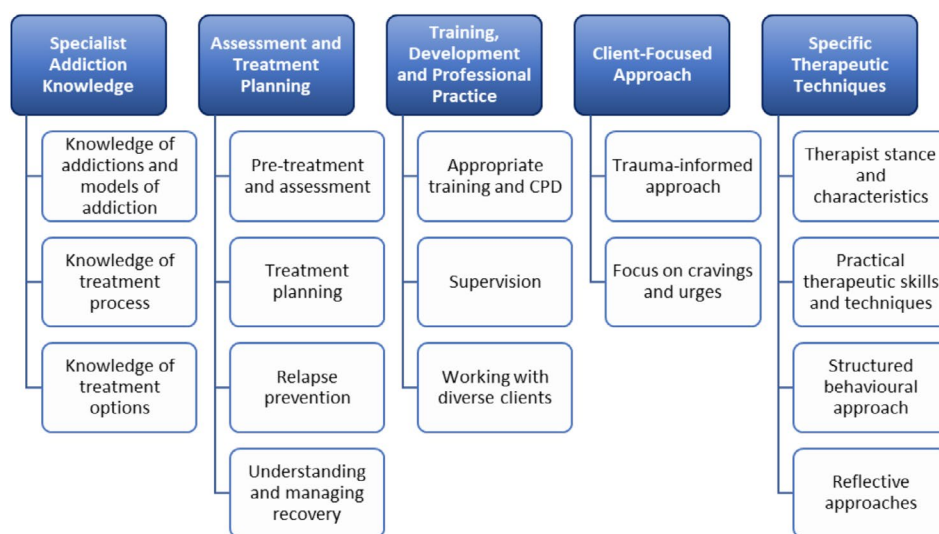


FIGURE 2 | Superordinate themes and sub-themes.

4.1.2 | Subtheme 2: Knowledge of Treatment Process

Therapists need a clear understanding of the treatment process that supports client recovery. The literature reviewed in this study identified staged intervention skills, including screening, assessment, treatment planning, counselling strategies, prevention, and pharmacological support (Pavlick et al. 2009; Lee et al. 2013).

4.1.3 | Subtheme 3: Knowledge of Treatment Options

The evidence suggested that therapeutic work is enhanced when therapists understand the local and national addiction services and treatment options available for their clients, including community-based programmes, residential rehabilitation, detox services, addiction-specific support groups, family therapy, and mutual aid initiatives (Graham et al. 2016; Steinglass 2009). Familiarity with medication-assisted treatments is desirable. Additionally, awareness of recovery-oriented and harm reduction approaches enables therapists to engage with contemporary best practices in addiction care. Therapists would benefit from familiarity with various treatment contexts, including outpatient and inpatient services, residential therapeutic communities, prisons, mental health services, community-based programmes, addiction-specific support services, rehabilitation and detox centres, and drop-in sessions (Gainsbury 2017; Graham et al. 2016). Understanding these diverse settings strengthens the ability to provide appropriate support and adapt interventions to meet clients' needs (Adrian 2001).

4.2 | Theme 2: Assessment and Treatment Planning

4.2.1 | Subtheme 1: Pre-Treatment Assessment

Good addiction therapy starts with well-structured, client-focused assessment. This process involves illuminating the client's history, physical and mental health, medications, symptom

and withdrawal symptom severity, housing and financial needs, family and social context, social isolation and potential risks to themselves or others (Brookes and Penn 2003; Clark 2009). A strengths-based assessment also identifies psychological characteristics, coping strategies and treatment readiness. Key considerations include obtaining informed consent, exploring alternative resources, and ensuring early engagement and safety in the therapeutic process, while also supporting clients to recognise the presence of addiction and addressing barriers to engagement (Clark 2009; Edwards and Loeb 2011).

4.2.2 | Subtheme 2: Treatment Planning

Robust treatment planning requires therapists and clients to work collaboratively to develop a plan tailored to the client's specific needs. Options may include structured, solution-focused interventions or more reflective, exploratory approaches based on client characteristics and treatment resources (Gottheil et al. 2002; Thornton et al. 2003). No matter the approach, effective treatment planning includes realistic goal setting, monitoring progress, and preparing for treatment completion and involves a multidisciplinary approach (Adrian 2001).

4.2.3 | Subtheme 3: Relapse Prevention

Relapse prevention is a critical aspect of addiction treatment in which therapists work with clients to assess and identify potential high-risk situations and responsive support strategies. Cravings, which are common yet unpredictable in recovery, pose a significant relapse risk (Joe et al. 2009; Lee et al. 2013). Relapse episodes can negatively impact self-image, triggering shame, diminished self-respect, and emotional distress, potentially leading to premature withdrawal from treatment/support (Weegmann and Khantzian 2011). Therapists are encouraged to develop skills in addressing these psychological effects, thus supporting clients in managing relapse-related experiences and reinforcing coping strategies to sustain recovery.

4.2.4 | Subtheme 4: Understanding and Managing Recovery

In addictions treatment, recovery is best approached holistically, emphasising client strengths, resources, and overall well-being (Weegmann and Khantzian 2011). Recovery is an individualised and dynamic process that may involve harm reduction, controlled substance reduction/use, or abstinence. Recognising clients' internal and external resources is helpful for effective treatment and lasting recovery (Krebs et al. 2018).

4.3 | Theme 3: Training, Development and Professional Practice

4.3.1 | Subtheme 1: Appropriate Training and CPD for Addictions Counsellors

Effective training and continuing professional development (CPD) are essential for therapists in addiction treatment (Lee 2014; Moro et al. 2016). Courses might want to cover substance use disorders, treatment planning, interdisciplinary approaches and evidence-based interventions, while also ensuring therapists can recognise co-occurring mental health and substance use conditions (Wheeler et al. 2014). Access to research opportunities is encouraged. The impact of counsellors' characteristics, beliefs, attitudes, and behaviours on treatment outcomes can be explored in reflective practice training.

4.3.2 | Subtheme 2: Supervision

Regular supervision with an addiction expert to promote specialist clinical skills and therapist self-awareness of stressors in addictions work is encouraged. Trauma-informed addiction supervision is also desirable. Maintaining a collaborative approach with supervisors helps therapists prioritise self-care, enhance professional development, and ensure responsible caseload management (Southern 2013). It is also recognised that peer-supervision can offer an effective support mechanism for therapists. Establishing a self-care support system for the therapist is essential to prevent burnout, while ongoing reflection on personal biases helps maintain ethical and effective practice (Giordano et al. 2016; Jones and Branco 2020).

4.3.3 | Subtheme 3: Working With Diverse Groups

Engagement and assessment in addictions counselling can be provided within a culturally sensitive framework that considers clients' backgrounds and readiness to change (Gainsbury 2017; Kasl 2002). Recognising and responding to issues of equality, diversity, and inclusion may be essential for ensuring a meaningful therapeutic relationship and effective treatment. Integrating cultural understanding could enhance awareness of diverse perspectives and responsiveness to clients' individual needs. Culturally competent practices, such as offering therapy in the client's first language and involving their family and community, have the potential to improve client engagement, retention, and outcomes (Gainsbury 2017; Steinglass 2009).

4.4 | Theme 4: Client-Focused Approach

4.4.1 | Subtheme 1: Trauma-Informed Approach

It is acknowledged there are strong connections between trauma and addiction (Giordano et al. 2016; Sannibale et al. 2013). A trauma-informed approach ensures treatment prioritises safety, trust, choice, collaboration, and empowerment. It integrates cognitive, behavioural, interpersonal, and case management strategies with a relational focus. Effective interventions include coping skills training, peer support, and psychoeducation to support clients in recovering from physical, sexual, and psychological trauma (Giordano et al. 2016; Spooner and Lyddon 2007).

4.4.2 | Subtheme 2: Focus on Cravings and Urges

Because cravings are unpredictable and often accompanied by strong physical sensations, they pose a significant relapse risk. Supporting clients in managing cravings and urges appears to be a critical component of addiction treatment (Kvamme et al. 2015; Pavlick et al. 2009). Effective strategies include coping skills training, psychoeducation on cravings and withdrawal symptoms, and mindfulness techniques (Vujanovic et al. 2017; Oakes et al. 2020).

4.5 | Theme 5: Specific Therapeutic Techniques

4.5.1 | Subtheme 1: Therapist Stance and Characteristics

In addiction therapy, therapists are urged to adopt a non-confrontational, empathic stance, particularly when working with clients who have experienced trauma (Jones et al. 2016; Allen and Olson 2016). Conveying hope, care, and acceptance is essential, along with maintaining flexibility rather than relying on rigid treatment styles. Key therapist qualities include openness, transparency, persistence, and patience, alongside compassion, authenticity, and a willingness to build mutual trust (Watts et al. 2018; Weegmann and Khantzian 2011).

4.5.2 | Subtheme 2: Practical Therapeutic Skills and Techniques

Therapeutic approaches such as motivational interviewing, solution-focused therapy, mindfulness, and mentalization-based therapy can support clients in addiction treatment (Lanza et al. 2014; Martino et al. 2006; Osborn 1997). These methods help address engagement, motivation, resistance, relational dynamics, dependency, and trauma. Key therapeutic skills include active and reflective listening, facilitative non-verbal behaviours, fostering human connection, and supporting healthy attachment. The awareness of transference and countertransference can provide valuable insights into relational patterns. Psychoeducation, goal setting, and experimental techniques can help clients develop emotional awareness and enhance creative capacities (Kasl 2002; Spooner and Lyddon 2007). A client-centred, collaborative, and strengths-based approach supports

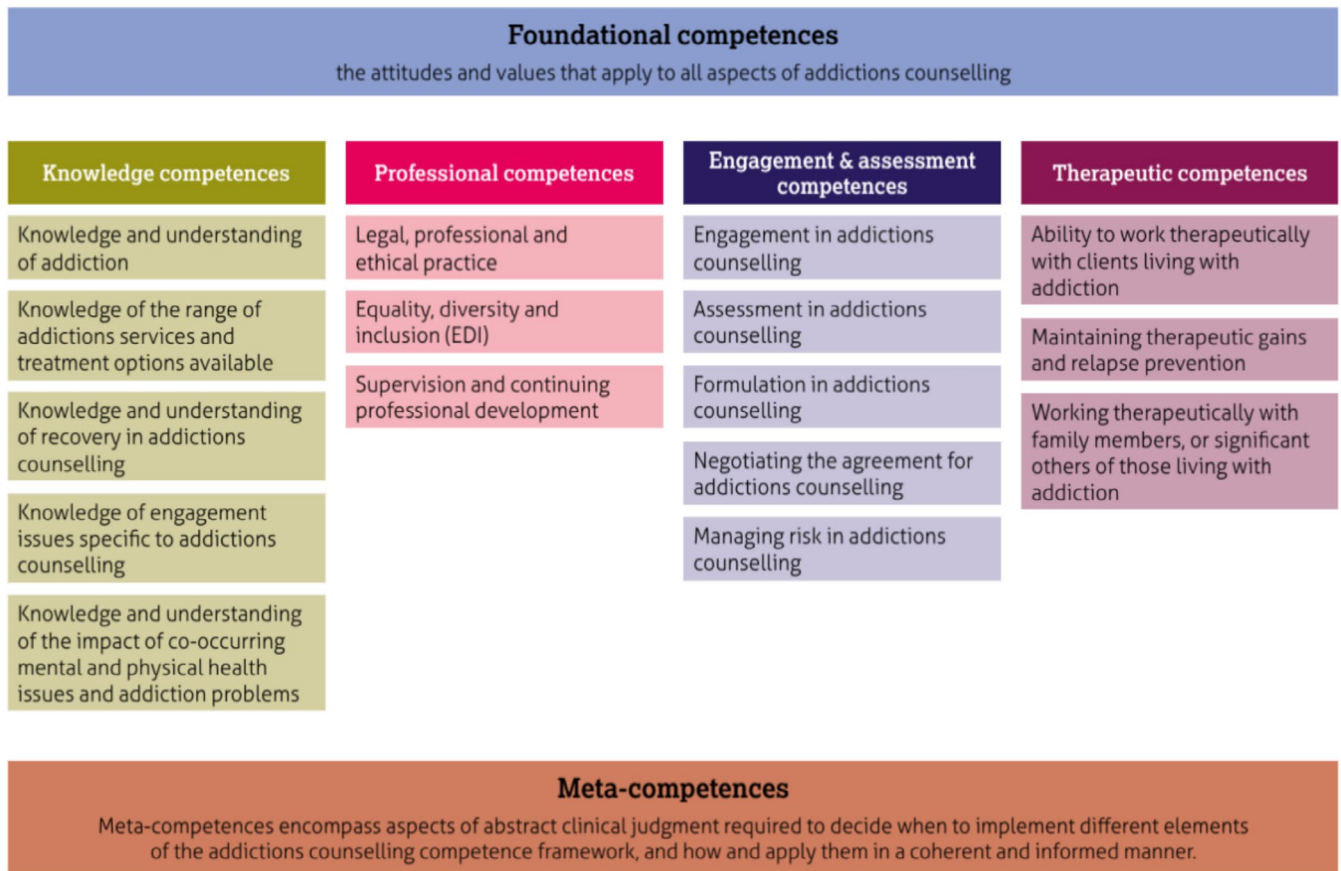


FIGURE 3 | The BACP's addictions competence framework (British Association for Counselling and Psychotherapy 2024).

long-term recovery and healing. Therapists should adopt flexible approaches to help clients manage low self-esteem, shame, and guilt, particularly in the context of relapse. Working with shame and self-respect in a relational manner is essential for long-term recovery (Weegmann and Khantzian 2011).

4.5.3 | Subtheme 3: Structured Behavioural Approaches

For some clients with addictions, structured behavioural approaches may be more effective. These approaches balance directive techniques for managing immediate cravings with facilitative methods that explore underlying emotional conflicts and physical and psychological dependencies (Thornton et al. 2003; Dimaggio et al. 2015). Key elements include maintaining an active stance, gathering specific information, providing direction and advice, reinforcing goal attainment, offering meaningful interpretations, and focusing on present interactions. Cognitive skills training helps challenge harmful thought patterns, while behavioural coping strategies support craving management and resistance to addiction (Dakwar and Levin 2013; Fraumeni-McBride 2019).

4.5.4 | Subtheme 4: Reflective Approaches

Reflective approaches emphasise a less directive stance, focusing on the client's emotions and underlying conflicts. These approaches encourage self-exploration and self-expression

by allowing silences, following the client's lead, and using practices that promote deeper self-reflection (Edwards and Loeb 2011; Spooner and Lyddon 2007). By creating a supportive and open therapeutic space, clients are encouraged to process their experiences at their own pace. A strong emphasis on the therapeutic relationship is considered a fundamental component of a reflective approach (Allen and Olson 2016; Jones et al. 2016).

This systematic literature review supported the development of the BACP's Addiction Competence Framework (Figure 3).

5 | Limitations

Article selection was limited to publications in English, which may have excluded some relevant research. Accessing one EBSCO database to undertake the literature search may have been a limiting factor. The use of other databases, such as Psych INFO, PubMed, Web of Science and EMBASE, may have extended the search strategy's reach. Unblinded screening and review may have introduced some bias into the final selection of studies, for example, if screeners were drawn to articles by well-known authors. One of the researchers involved in screening, selection and analysis was a senior addictions specialist. Other researchers may have deferred to their opinions and perspectives. The high number of sources initially generated from the search strategy (11,128 articles identified) led to most articles being excluded following screening of their titles and abstracts,

as irrelevant to the subject area. In hindsight, the review procedure might have benefitted from narrower search terms.

6 | Conclusion

Addiction counselling requires an integrative, evidence-based approach that combines specialist knowledge, structured assessment, ongoing professional development, client-focused care, and tailored therapeutic techniques (Clark 2009; Capuzzi and Stauffer 2024). By fostering a supportive, trauma-informed, and culturally responsive environment, counsellors can help individuals navigate addiction complexities and achieve meaningful, sustainable recovery (Giordano et al. 2016; Gainsbury 2017).

Specialist knowledge is fundamental for understanding addiction models, treatment processes, and therapeutic contexts (Thombs and Osborne 2001; Lee et al. 2013). Therapists are encouraged to be well-versed in the biological, psychological, and social dimensions of addiction, including co-occurring disorders and systemic influences. Familiarity with various treatment services and approaches is essential to ensure clients receive appropriate, tailored care (Lanza et al. 2014; Steinglass 2009).

Assessment and treatment planning are crucial in addiction recovery. Effective pre-treatment assessment evaluates a client's history, symptom severity, and readiness for change. Treatment planning structures interventions to be responsive and engaging, and integrates relapse prevention strategies to address psychological and physiological factors while reinforcing positive coping mechanisms (Pavlick et al. 2009; Kvamme et al. 2015).

Training, development, and professional practice are significant in addiction counselling. Continuous professional development keeps practitioners updated on emerging research and treatment models (Moro et al. 2016). Supervision helps maintain ethical standards, prevent burnout, and foster professional growth. Cultural awareness is essential to adapt interventions to meet the unique needs of diverse populations (Gainsbury 2017; Kasl 2002).

A client-focused approach recognises that treatment can be adapted to each individual's circumstances. A trauma-informed perspective is particularly important, given the prevalence of trauma among those with addiction (Giordano et al. 2016; Weegmann and Khantzian 2011). Therapists are encouraged to explore underlying emotional factors, like shame, guilt, and self-esteem issues, that impact relapse risk and overall well-being. The therapeutic process should be collaborative, empowering clients to take an active role in their recovery while fostering resilience and self-efficacy.

The application of specific therapeutic techniques is crucial to align interventions with client needs. A therapist's stance, characterised by empathy, honesty, and flexibility, significantly impacts treatment effectiveness (Allen and Olson 2016).

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Endnotes

¹ In this article, we use the generic term 'therapists' to refer to counsellors, psychotherapists, and other talking therapy practitioners.

² Addiction, Substance Use, Substance Misuse, Alcohol, Drugs, Gambling, Sex, Porn, Criminal Justice, Abuse, Dual Diagnosis, Psychoeducation, Recovery, Relapse, SMART, 12 Step, NA (Narcotics Anonymous), AA (Alcoholics Anonymous), Therapeutic Communities, Prison, Trauma.

³ Counselling (Counsel), Psychotherapy (Psychothera), Therapy, Mental Health, Psychology, Person Centred, Humanistic, Psychodynamic, CBT, Psychoanalytical, Gestalt, Existential, TA, Behavioural (Behavio*), CAT, Motivational Interviewing, Pharmacology, Ethnicity, Diversity, Sexuality, Trans-theoretical, Mindfulness, Treatment, Support, Service, Agencies, Care Pathways, Third Sector, Private Practice, NHS, IAPT.

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